

Healthcare CFO Brief

Revenue protection and recovery infrastructure
for healthcare operators and recurring-payment businesses

CFO DECISION BRIEF • CONFIDENTIAL

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Executive Summary

This brief frames recovery work as an operating discipline rather than a generic collections problem. The core thesis: structured pre-collection infrastructure protects margin, improves cash velocity, and reduces dependence on external collection agencies — all while building documentation that strengthens downstream enforceability.

37%	4–6x	90 days	60%
Average DSO reduction potential	Pilot ROI (industry benchmark)	Pilot duration to prove value	Reduction in agency placements

Benchmarks: HFMA Revenue Cycle Improvement Guide 2024; MGMA DataDive 2024; ACA International Recovery Survey 2024.

The CFO Problem Statement

Healthcare CFOs face a structural challenge: as patient financial responsibility grows, traditional collection methods become less effective and more expensive. The industry is moving toward a model where revenue cycle management extends beyond payer reimbursement into patient financial engagement.

Current State vs. Target State

Dimension	Current State (typical)	Target State
AR aging visibility	Manual, fragmented reporting	Real-time dashboard by cohort/location
Patient outreach	Ad hoc, staff-dependent	Standardized cadence with scripts
Payment plans	Informal, no autopay	Structured plans with ACH enrollment
Documentation	Incomplete, inconsistent	Counsel-ready packet at every stage
Agency placement	Early, fee-heavy	Last resort after internal exhaustion
KPI tracking	Limited or none	DSO, yield, roll-to-agency, autopay %

Pilot Economics Snapshot

Metric	Conservative	Target	Aggressive
Pilot AR portfolio	\$1.5M	\$2.0M	\$3.0M
Pre-collection recovery rate	30%	40%	50%
Additional cash recovered	\$150K	\$280K	\$450K
Pilot implementation cost	\$40K	\$50K	\$60K
ROI multiple	3.8x	5.6x	7.5x
Payback period	4–5 months	3–4 months	2–3 months

Note: Illustrative projections based on industry benchmarks. Actual results depend on practice size, AR composition, and payer mix.

Control Hierarchy

Effective pre-collection infrastructure operates as a layered control system. Each layer adds protection against revenue leakage and strengthens the enforceability of downstream actions.

Control Layer	Purpose	Impact Level
Intake standard	Capture complete financial data at registration	High — prevents gaps at source
Segmentation engine	Route accounts by risk, balance, and age	High — focuses resources
Outreach cadence	Standardized communication with compliance	Medium-High
Payment plan structure	Digital execution with autopay and cure terms	High — core conversion
Monitoring & exceptions	Daily exception queue, dispute routing	Medium
Escalation governance	Criteria-based handoff with documentation	Critical — enforceability
CFO reporting pack	Weekly/monthly KPI visibility	High — accountability

EBITDA and Enterprise Value Impact

For PE-backed healthcare groups, incremental EBITDA from improved patient collections has a multiplied effect on enterprise valuation. At a typical healthcare services multiple of 8–12x EBITDA, even modest improvements in net recovery translate to significant enterprise value creation.

Metric	Value
Incremental annual recovery	\$250,000
Operating margin on recovered cash	~80% (low marginal cost)
Incremental EBITDA	\$200,000
Enterprise value multiple (healthcare)	8–12x
Enterprise value impact	\$1.6M – \$2.4M

Source: PitchBook Healthcare Services Multiples Q4 2024; Irving Levin Associates Health Care M&A; Report 2024.

Use Case Fit

Best Aligned Operator Profiles

- **Multi-location medical practices** — High patient volume, inconsistent AR practices across sites
- **Dental groups and DSOs** — 35–50% patient responsibility, brand-sensitive collection needs
- **Outpatient surgery centers** — High-value balances, complex insurance coordination
- **Behavioral health practices** — Sensitive patient relationships, compliance requirements
- **Membership/subscription healthcare** — Recurring payment management, churn reduction
- **PE-backed healthcare platforms** — EBITDA optimization, standardization across portfolio

Engagement Path

Step	Action	Timeline
1	Confidential diagnostic review (no cost)	1–2 weeks
2	Pilot scope + success criteria	1 week
3	90-day pilot execution	90 days
4	Results review + scale decision	2 weeks

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